

Medicare — Educational Planning Guide

A clean, plain-English walkthrough of what Medicare covers, what it doesn't cover, the only two ways to structure coverage, and the enrollment deadlines that shape informed decisions.

The Big Picture

- Medicare is federal health insurance (primarily for age 65+, certain disabilities, ESRD).
- Original Medicare = Part A (hospital) + Part B (medical).
- Part D is optional prescription drug coverage offered by private plans approved by Medicare.
- Part C (Medicare Advantage) is the private-plan alternative that replaces how you receive Part A & Part B benefits (often includes Part D).
- There are only two common ways people build coverage:
 - 1) Original Medicare (A+B) + Medicare Supplement (Medigap) + (usually) Part D
 - 2) Medicare Advantage (Part C) (often bundled with Part D)

What Each Part Covers (and what people confuse)

Part	What it generally covers	Common surprises / misunderstandings
Part A	Inpatient hospital care, skilled nursing facility care (limited), hospice, some home health.	"Skilled nursing" ≠ long-term custodial care. Benefit periods and day limits apply.
Part B	Doctor services, outpatient care, preventive services, ambulance (medically necessary), durable medical equipment (DME), limited outpatient drugs in specific settings.	Part B is where many ongoing premiums live; late enrollment penalties are common.
Part D	Prescription drugs through a plan formulary; cost-sharing depends on the plan, pharmacy, and coverage phase.	Even if you take no meds today, delaying can trigger a lifetime penalty if you go too long without creditable coverage.

Part	What it generally covers	Common surprises / misunderstandings
Part C	A private plan that must cover Part A & Part B services. Most plans include Part D and may include extra benefits (vision, hearing, dental), with network rules.	Networks, prior authorization, and plan rules can shape access and out-of-pocket costs.

What the Government Will Cover vs. Not Cover (Original Medicare)

- Generally covered: medically necessary inpatient care (Part A) and outpatient/physician care (Part B), with deductibles, coinsurance, and coverage rules.
- Commonly not covered: most dental care, routine vision/hearing, long-term custodial care (assistance with daily living), routine foot care, and most care outside the U.S.
- Medicare may cover short-term skilled rehabilitation after a qualifying hospital stay, but not ongoing custodial long-term care.

The Only Two Ways to Structure Medicare Coverage

- Path 1: Original Medicare + Medigap + (usually) Part D.
- You keep Original Medicare as your primary coverage (Parts A & B).
- You add a Medicare Supplement (Medigap) policy to help pay some of the out-of-pocket costs Medicare leaves behind.
- You typically add a standalone Part D plan for prescriptions.
- Considerations include provider flexibility, predictable cost-sharing, and fewer network constraints.
- Path 2: Medicare Advantage (Part C).
- You enroll in a private Medicare-approved plan that delivers your Part A & B benefits.
- Most plans include Part D and may include additional benefits such as vision, hearing, or dental.
- Considerations include network structure, prior authorization requirements, and plan-specific rules.

Enrollment Periods — the deadlines most people miss

- Initial Enrollment Period (IEP): a 7-month window around turning 65.
- General Enrollment Period (GEP): Jan 1 – Mar 31 each year if IEP was missed.
- Annual Open Enrollment (AEP): Oct 15 – Dec 7 to review or change Medicare Advantage or Part D coverage.
- Medicare Advantage Open Enrollment Period (MA OEP): Jan 1 – Mar 31 for individuals already enrolled in Medicare Advantage.
- Special Enrollment Periods (SEPs): triggered by qualifying life events such as relocation or loss of employer coverage.
- Medigap Open Enrollment: a one-time 6-month window beginning when age 65+ and enrolled in Part B.

Late Enrollment Penalties (why timing matters)

- Part B penalty: typically 10% for each full 12-month period enrollment was delayed.
- Part D penalty: typically 1% per month without creditable prescription coverage for 63+ days.

Using This Information to Make an Informed Decision

- Confirm eligibility timing and when Medicare coverage begins.
- Clarify priorities such as provider access, cost predictability, or integrated plan design.
- Understand current coverage (employer, retiree, VA, TRICARE, Medicaid) and how it coordinates with Medicare.
- Evaluate the two structural approaches: A+B+Medigap+(Part D) versus Part C.
- Identify coverage gaps Medicare does not address, such as dental, vision, hearing, and custodial care.
- Note applicable enrollment deadlines and protections.
- Document practical considerations including providers, medications, travel habits, and budget tolerance.

What to gather before selecting coverage (Education Checklist)

- Preferred doctors and hospitals.
- Medication list and preferred pharmacy.
- Chronic conditions or anticipated procedures.
- Travel patterns (in-state, out-of-state, international).
- Budget tolerance: premium vs. point-of-service exposure.
- Employer or union coverage status.

Protect your health. Protect your wealth. Protect your family. Protect your legacy.

DISCLOSURE: This guide is for educational purposes only and isn't legal or tax advice. Medicare rules can change, and plan availability, benefits, provider networks, and costs vary by county and carrier.